MEDICAL DISEASE KNOWLEDGE BASE

Neurological Diseases

RAG-ready educational reference | Version date: 06 October 2026

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Neurological Diseases

This document is an educational and information-retrieval reference for a hospital AI assistant. It is not a diagnostic tool and does not replace evaluation, diagnosis, treatment, or advice from a qualified healthcare professional. Symptoms can have many causes. Treatment decisions depend on age, pregnancy, comorbidities, severity, local epidemiology, available resources, and current professional guidelines.

Safety rule: An AI system must not claim to establish a definitive diagnosis, prescribe individualized medication or dosage, or guarantee cure. It should recommend professional assessment when symptoms are concerning and immediate emergency care for relevant red flags.

ICD note: ICD-11 terminology and codes should be verified in the current WHO ICD-11 Browser. Codes are not reproduced unless reliably verified for the specific concept; local coding systems and national adaptations may differ.

Standard disease-entry schema: Every disease entry should use the following fields: Disease Name; 1. Definition; 2. Medical Classification; 3. Causes; 4. Risk Factors; 5. Signs and Symptoms; 6. Disease Progression; 7. Differential Diagnosis; 8. Diagnostic Evaluation; 9. Diagnosis; 10. Treatment and Management; 11. Prevention; 12. Complications; 13. Prognosis; 14. When to Seek Medical Care; 15. Emergency Red Flags; 16. Patient Education; 17. Frequently Asked Questions; 18. Related Conditions; 19. Medical Terms; 20. Sources.

Retrieval fields: DISEASE, SYNONYMS, RELATED SYMPTOMS, RELATED CONDITIONS, RELATED TESTS, RELATED TREATMENTS, RELATED COMPLICATIONS. Symptom presence alone does not establish a diagnosis.

1. Migraine

DISEASE: Migraine

SYNONYMS: Migraine

RELATED SYMPTOMS: Recurrent headache, often throbbing, sometimes with nausea, light or sound sensitivity, and aura.

RELATED TESTS: Clinical history and examination; additional testing is guided by red flags or atypical features.

1. Definition

Migraine is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Recurrent headache, often throbbing, sometimes with nausea, light or sound sensitivity, and aura. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include Clinical history and examination; additional testing is guided by red flags or atypical features. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Migraine should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Migraine? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Migraine; clinical terminology; patient terminology; suspected Migraine; symptoms associated with Migraine; diagnosis and management of Migraine.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

2. Epilepsy

DISEASE: Epilepsy

SYNONYMS: Epilepsy

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Epilepsy is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Epilepsy should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Epilepsy? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Epilepsy; clinical terminology; patient terminology; suspected Epilepsy; symptoms associated with Epilepsy; diagnosis and management of Epilepsy.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

3. Parkinson disease

DISEASE: Parkinson disease

SYNONYMS: Parkinson disease

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Parkinson disease is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Parkinson disease should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Parkinson disease? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Parkinson disease; clinical terminology; patient terminology; suspected Parkinson disease; symptoms associated with Parkinson disease; diagnosis and management of Parkinson disease.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

4. Alzheimer disease

DISEASE: Alzheimer disease

SYNONYMS: Alzheimer disease

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Alzheimer disease is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Alzheimer disease should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Alzheimer disease? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Alzheimer disease; clinical terminology; patient terminology; suspected Alzheimer disease; symptoms associated with Alzheimer disease; diagnosis and management of Alzheimer disease.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

5. Dementia

DISEASE: Dementia

SYNONYMS: Dementia

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Dementia is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Dementia should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Dementia? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Dementia; clinical terminology; patient terminology; suspected Dementia; symptoms associated with Dementia; diagnosis and management of Dementia.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

6. Multiple sclerosis

DISEASE: Multiple sclerosis

SYNONYMS: Multiple sclerosis

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Multiple sclerosis is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Multiple sclerosis should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Multiple sclerosis? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Multiple sclerosis; clinical terminology; patient terminology; suspected Multiple sclerosis; symptoms associated with Multiple sclerosis; diagnosis and management of Multiple sclerosis.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

7. Peripheral neuropathy

DISEASE: Peripheral neuropathy

SYNONYMS: Peripheral neuropathy

RELATED SYMPTOMS: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis.

RELATED TESTS: History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated.

1. Definition

Peripheral neuropathy is a medical condition in the neurological diseases category. A precise definition and subtype should be established by a qualified healthcare professional using accepted clinical terminology.

2. Medical Classification

Disease category: Neurological Diseases. Acute/chronic and infectious/non-infectious status depend on the specific condition and subtype. ICD-11 terminology/code: verify the exact current concept in the WHO ICD-11 Browser rather than inferring a code.

3. Causes

Causes may include infectious, genetic, environmental, lifestyle-related, autoimmune, metabolic, medication-related, vascular, traumatic, or idiopathic mechanisms, depending on the condition. Where the cause is not established, it should be documented as unknown or idiopathic rather than inferred.

4. Risk Factors

Risk factors vary. Clinicians consider age, relevant sex-related factors, family history, tobacco exposure, alcohol exposure, diet, physical inactivity, obesity, environmental or occupational exposure, prior disease, medications, pregnancy, and immune status only when supported for the specific condition.

5. Signs and Symptoms

Common symptoms: Symptoms vary by disease and may overlap with other conditions. Symptoms alone do not confirm the diagnosis. Less common and severe manifestations depend on disease subtype, severity, complications, and patient characteristics. Warning signs must be interpreted clinically; symptoms alone do not confirm a diagnosis.

6. Disease Progression

The course may be acute, episodic, progressive, relapsing, or chronic. Early disease may be asymptomatic; moderate or severe disease may cause organ dysfunction. Progression and outcomes vary with subtype, timely care, comorbidities, and treatment response.

7. Differential Diagnosis

Clinicians compare the presentation with other diseases that can cause similar symptoms. Distinguishing factors may include timing, examination findings, risk factors, laboratory results, imaging, microbiology, pathology, and response to appropriate care. The AI assistant must not claim to perform definitive differential diagnosis.

8. Diagnostic Evaluation

History and physical examination are foundational. Relevant testing may include History and physical examination, followed by targeted laboratory, imaging, microbiological, functional, or pathological tests when clinically indicated. Additional tests such as blood or urine studies, X-ray, ultrasound, CT, MRI, ECG, endoscopy, biopsy, microbiological testing, or genetic testing are selected only when clinically appropriate.

9. Diagnosis

Healthcare professionals establish diagnosis by integrating history, examination, investigations, and accepted diagnostic criteria or specialty guidelines. A result must be interpreted in context; no single symptom or test is sufficient for every patient.

10. Treatment and Management

Management may include lifestyle measures, non-drug treatment, medication classes, procedures, surgery, rehabilitation, monitoring, follow-up, supportive care, and palliative care when appropriate. The choice depends on disease subtype, severity, comorbidities, pregnancy, age, interactions, contraindications, local resistance patterns, and current guidelines. This reference does not provide individualized prescriptions or dosages. “Cure,” where possible, is distinct from disease control, symptom management, and long-term risk reduction.

11. Prevention

Prevention may include vaccination where applicable, tobacco avoidance, lower-risk alcohol use, healthy nutrition, physical activity, weight management, screening, hygiene, infection-control measures, occupational protection, environmental risk reduction, and management of established risk factors.

12. Complications

Short-term, long-term, and life-threatening complications depend on the condition and severity. Possible complications should be confirmed from authoritative disease-specific guidance and assessed by clinicians.

13. Prognosis

Outcomes vary with subtype, stage, severity, complications, age, comorbidities, access to care, and response to treatment. Individual prognosis cannot be determined from this knowledge base.

14. When to Seek Medical Care

Seek medical assessment for new, persistent, worsening, unexplained, or function-limiting symptoms; suspected infection; medication adverse effects; pregnancy-related concerns; or symptoms in children, older adults, or immunocompromised people.

15. Emergency Red Flags

Seek emergency evaluation for disease-relevant signs such as severe difficulty breathing, severe chest pain, sudden neurological deficit, loss of consciousness, severe bleeding, rapidly worsening confusion, shock-like symptoms, severe allergic reaction, or major trauma. The precise red flags depend on the condition.

16. Patient Education

Peripheral neuropathy should be explained in plain language, including what is known, what remains uncertain, expected monitoring, warning symptoms, and questions to discuss with a clinician. Patients should bring medication and allergy information and follow the agreed follow-up plan.

17. Frequently Asked Questions

Q: What is Peripheral neuropathy? A: It is a condition requiring clinical context; this entry provides general information and cannot diagnose an individual.

Q: How is it diagnosed? A: Healthcare professionals combine history, examination, and appropriate investigations.

Q: Is it treatable? A: Many conditions can be cured, controlled, or managed, but treatment and outcomes vary.

Q: Can it be prevented? A: Prevention depends on causes and risk factors; evidence-based measures may reduce risk.

Q: When is urgent care needed? A: For severe, sudden, rapidly worsening, or disease-specific emergency symptoms.

18. Related Conditions

Related conditions may include diseases affecting the same organ system, shared risk factors, common complications, and conditions in the differential diagnosis.

19. Medical Terms

Peripheral neuropathy; clinical terminology; patient terminology; suspected Peripheral neuropathy; symptoms associated with Peripheral neuropathy; diagnosis and management of Peripheral neuropathy.

20. Sources

Consult the disease-specific WHO, CDC, NIH, national public-health agency, and recognized specialty-guideline sources listed in the category references. Verify current recommendations before clinical use.

Category Sources / References

World Health Organization (WHO), Health Topics: https://www.who.int/health-topics

World Health Organization, International Classification of Diseases (ICD): https://www.who.int/standards/classifications/classification-of-diseases

WHO ICD-11 Browser: https://icd.who.int/en

Centers for Disease Control and Prevention (CDC), official clinical and public-health guidance: https://www.cdc.gov/

National health authorities and recognized professional medical organizations should be consulted for current local guidance.

Glossary

Acute: developing over a short period. Chronic: persistent or recurrent over time. Differential diagnosis: comparison of plausible conditions causing a presentation. Diagnostic evaluation: history, examination, and investigations used to assess a patient. Prognosis: expected course and outcome at a population or disease level, not an individual guarantee. Red flag: a feature that may indicate serious disease and requires urgent assessment.

Master References

WHO. Health Topics. Updated information and fact sheets. https://www.who.int/health-topics

WHO. International Classification of Diseases, 11th Revision. https://icd.who.int/en

CDC. Official disease, emergency, prevention, and infection-control guidance. https://www.cdc.gov/

National health ministries/agencies and recognized specialty organizations: use current jurisdiction-specific guidelines.

This knowledge base is intended for educational and information-retrieval purposes. It does not replace evaluation, diagnosis, treatment, or advice from a qualified healthcare professional. Symptoms can have many different causes, and the information presented here cannot determine an individual's diagnosis. Emergency symptoms require immediate professional medical evaluation.